

What Do You Need To Hear? (Mis)alignment in Sympathy Expressions

ABSTRACT

Grief is a prevalent, consequential, and underexamined experience in organizational life. Despite the ubiquity of grief at work, people often do not know what to say when someone is grieving. Integrating the Interpersonal Emotion Regulation Literature with Basic Psychological Needs Theory, the current research reveals a paradox. When sympathizers communicate with those who are grieving, they systematically prioritize changing the griever's feelings by positively reframing their experiences over validating them, even when doing so undermines the very goal they are trying to accomplish (i.e., alleviating distress). Prioritizing a positive reframe backfires because it is misaligned with what grievers need to hear – a message that makes them feel understood. However, positive reframing does not always backfire. When loss events thwart one's need for esteem (e.g., job loss), positive reframing instead becomes the more effective strategy. Across an archival analysis of over 600 sympathy cards, surveys of over 1,000 people consoling someone in their grief (including over 150 medical professionals), and three experiments with over 1,500 active grievers, I find support for this nuanced paradox. Theoretically, this research expands Basic Psychological Needs Theory by taking need satisfaction out from *within* individuals and into the rich interplay of interpersonal communication *between* them. Practically, this research has important implications for sympathy communication at work, as well as the role that training may play in facilitating need-message alignment.

Keywords:

grief, validation, reframing, interpersonal emotion regulation, psychological needs

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Grappling with what to say when someone is grieving is common (Petriglieri & Maitlis, 2019). After all, what can you say when the unthinkable has happened to someone you know? On the surface, it would appear that what people say when they express sympathy is inconsequential; grief – a prevalent experience as one in four employees will experience loss in any given year (Dutton et al., 2014) – is a profound, often destabilizing emotional response to loss (Stroebe, 1993), and no special phrase can bring back a loved one, mend a broken heart, solve one's financial difficulties, or cure one's illness (Calderwood & Alberton, 2024). Yet, people reliably turn to others for comfort through conversation (Rack et al., 2008), and the words chosen in these moments can shape whether support alleviates or inadvertently deepens distress.

In the current research, I investigate three interrelated questions: When expressing sympathy, which communication strategies do people rely on, and what motivates the selection of these strategies? And what is actually effective in alleviating another's distress? In answering these questions, I observe a paradox: When people communicate with those who are grieving, they systematically prioritize changing the griever's feelings by expressing a positive reframe instead of validation, even when doing so undermines the very goal they are trying to accomplish (i.e., alleviating distress). Moreover, empathic concern *positively* predicts positive reframing messages, suggesting that greater concern for grievers may ironically lead to less effective messages. Finally, I find that positive reframing does not always backfire, demonstrating that the psychological need that is thwarted from the loss – i.e., one's need for relatedness versus one's need for esteem – moderates whether validation or positive reframing effectively alleviates a griever's distress. Together, these results point to a need-message alignment framework for supportive communication.

Broadly speaking, this research makes three primary contributions to the literature (see General Discussion). First, I contribute to compassion theory (Dutton et al., 2014) by unpacking the ‘black box’ of what people communicate as a form of compassionate action. In doing so, I show that some of the most common ways of enacting compassion may be at odds with what is most effective at alleviating suffering. Second, I expand the interpersonal emotion regulation literature (Niven et al., 2024) by showing that errors in supportive communication may often come from those who are the *most* empathically concerned. Third, by showing that the thwarted psychological need from the loss event moderates whether positive reframing or validation is more effective, I expand Basic Psychological Needs Theory (Dweck, 2017; Ryan & Deci, 2008) by taking need satisfaction out from *within* individuals and into the rich interplay of interpersonal communication *between* them.

Interpersonal Emotion Regulation

What do people say when expressing sympathy? Research and theory from the extrinsic interpersonal emotion regulation literature (Zaki & Williams, 2013) offers some insights. Extrinsic interpersonal emotion regulation is defined as the formation and pursuit of goals designed to change others’ emotions through social interactions (Zaki, 2020). The process begins with the sympathizer evaluating the divergence between the (inferred) current emotional state of the griever (e.g., “they must feel really down”) and a desired emotional state for them (e.g., “they need to be brought back up again”). Once the sympathizer decides to regulate the target’s emotions, they must then select an appropriate means (i.e., a strategy) to do so. Although strategy selection has been extensively studied in the space of regulating one’s own emotions (i.e., intrapersonal emotion regulation; Gross, 2015), there has been relatively little research in the

context of regulating others' emotions (Pauw et al., 2025). However, the literature has identified some common ways in which people try to modify the emotions of another.

One popular strategy is to express a positive reframe (Matthews et al., 2022; Sahi et al., 2025), defined as shifting one's perspective to focus on the potential positive aspects, benefits, or outcomes of their loss (Niven et al., 2009; Nozaki & Mikolajczak, 2023). There are a number of ways that people can point to the 'silver lining' in one's difficulty (Petrova & Gross, 2023), such as by helping them adopt a temporally distanced perspective by emphasizing how things can change over time (e.g., "you'll get better"), helping them focus more on the big picture and other positive experiences in their life (e.g., "you still have so much life ahead of you"), and by helping them expand their perspective by considering how others might be thinking or feeling in the situation (e.g., "your loved one would want you to move on and live a happy life"; Sahi et al., 2023). Overall, positive reframing improves the way people *feel* about an event by changing the way they *think* about it (Jurkiewicz et al., 2023).

On the other hand, sympathizers may instead adopt a strategy that does *not* try to change the emotions of another. Validation is defined as expressing an understanding of one's experiences without directly trying to change the way they feel (Bradley et al., 2024). Expressions of validation contextualize (e.g., "it makes sense that you feel angry"), normalize (e.g., "anyone in your shoes would feel the same"), and express an understanding of (e.g., "you must be overwhelmed having to deal with this") one's emotions and experiences (Fleck, 2025). Validation helps people to feel understood and accepted (Reis et al., 2004), thereby creating an atmosphere where people feel safe to process their emotions (Linehan, 1997). Positive reframing and validation are similar in the sense that both strategies require the sympathizer to meaningfully engage with the griever's emotions, but they differ considerably in the extent to

which the strategy is aimed at altering their emotions (Bradley et al., 2024). When it comes to sympathy expressions, or explicit communication in response to another's loss (Koopmann-Holm & Tsai, 2014), sympathizers often face a conflict – to regulate or recognize, to alter or acknowledge, to uplift or sit with, to reframe or validate. Which of these two strategies might be more prevalent?

Previous data and theory point to a strong preference for communicating a positive reframe. First, a positive reframe may be the more normative thing to express. Especially in North American contexts, people reward and value high-arousal positive states such as excitement and enthusiasm (Ford, 2025) – a preference that shows up in everything from children's storybooks to national leaders' smiles (Tsai, 2017). This preference becomes crystalized in the workplace; organizations codify this positivity bias through display rules, rewarding upbeat emotion displays and subtly penalizing negative ones, like sadness or anger (Diefendorff & Richard, 2003). Indeed, the preference for positivity may become so strong as to become 'toxic', with recent work noting that employees often feel pressured to "gloss over" hardship rather than acknowledge it (Goodman, 2022).

The prevalence of positive reframing may also be due to self-oriented, egocentric reasons. Focusing on the positive allows sympathizers to shift out of difficult, uncomfortable conversations (Maynard, 1997). Focusing on the positive may also make the sympathizer feel better. Research suggests that attempting to improve another's emotions (e.g., through positive reframing) also has benefits for regulators themselves, as such regulatory experiences increase one's own positive emotions due to the 'warm glow' of engaging in prosocial action (Jurkiewicz & Oveis, 2024).

It is also difficult, and interpersonally risky (Yu et al., 2021), to validate another's experiences. It takes therapists years of training to be able to sit with another's pain with "unconditional acceptance" (Rogers, 1957). Indeed, medical residents in oncology must receive explicit training in expressing validation when breaking bad news to patients and their families (e.g., SPIKES protocol; Baile et al., 2000). And, when people do acknowledge distress, they underestimate its effectiveness in alleviating it (Torre & Lieberman, 2018). For instance, when asked on a trial-by-trial basis how much distress they would feel in response to an aversive image, participants correctly predicted that they would feel less distress if they changed how they thought about it but incorrectly predicted that they would feel *more* distress if they labelled and acknowledged their emotions (Lieberman et al., 2011). To sympathizers, it may therefore seem counterintuitive that accepting, as opposed to trying to change, another's difficulty indirectly alleviates their suffering. For these reasons, I hypothesize:

Hypothesis 1: Positive reframing is more prevalent than expressions of validation.

Indeed, some research finds related support for this hypothesis (Nozaki & Gross, 2025). For instance, in one study (Horowitz et al., 2001), when participants were asked how they would respond to someone who was emotionally distressed, most tried to 'fix' their problem (e.g., through offering advice) instead of expressing an understanding of their situation. As previously noted, the motivational antecedents for such a "change bias" (Bradley et al., 2024) are often theorized to be egocentric or normative in nature. In the current research, however, I challenge this assumption and offer an alternative explanation for why sympathizers may be more motivated to express a positive reframe instead of validation.

Positive Reframing is Motivated by Feelings of Compassion

Decades of research in psychology and management has demonstrated the other-oriented benefits of feeling empathic concern, or “other-oriented feelings that are most often congruent with the perceived welfare of the other person” (Batson, 1994: 606). For instance, people with greater empathic concern are less likely to morally disengage at work (Ogunfowora et al., 2022) and they are more likely to engage in organizational citizenship behaviors (Kamdar et al., 2006; Settoon & Mossholder, 2002). In fact, people who are empathically concerned are so other-oriented that they become emotionally exhausted from helping others (Lin et al., 2022). In short, a wealth of research gives the impression that those who are most empathically concerned will express sympathy in a way that is highly person-centered and supportive (Burleson, 2009).

According to the *empathy-altruism hypothesis* (Batson et al., 2023), however, empathic concern activates an approach-oriented, altruistic goal to alleviate another’s suffering and improve their welfare. For instance, in the classic studies by Batson et al. (1983), participants who felt greater empathic concern as opposed to personal distress (a self-oriented emotional reaction of discomfort and distress when witnessing another person’s suffering) were more likely to replace themselves with an ostensible research participant who was distressed after receiving electric shocks. Although empathic concern has never been explicitly linked to how people communicate support (Nozaki & Gross, 2025), this suggests that empathically concerned sympathizers will be most motivated to try to *change* the way that a griever feels, because doing so directly serves their altruistic intentions. That is, because positive reframing is an active attempt to alleviate another’s suffering (e.g., “look on the bright side, your loved one isn’t suffering any more”), whereas validation does not offer any clear mechanism for change (e.g., “this sucks”), positive reframing may seem more action-oriented and altruistic. Put formally:

Hypothesis 2: Empathic concern predicts a greater probability of expressing positive reframing over validation.

A Psychological Needs-Message Alignment Perspective

Basic psychological needs, or the “fundamental nutrients or supports that individuals must have to thrive” (Ryan & Deci, 2017: 80) have a long history in the psychological (e.g., Murray, 1938) and organizational (e.g., Deci et al., 1989) literature. At the most basic level, the theory proposes that when people have their needs met, they flourish; when people have their needs thwarted, they flounder (Ryan & Deci, 2017). Considering that grief has such a profound negative impact on one’s well-being (Bento, 1994), a salient question emerges: What psychological needs are thwarted from this adverse event, and what sympathy expressions may satisfy this need?

I argue that, despite being less prevalent, expressions of validation are more effective than expressions of positive reframing because they are more likely to meet one’s need for relatedness (Ryan & Deci, 2017; also called acceptance; Dweck, 2017). The need for relatedness pertains to feeling that one is close and connected to others (Reis et al., 2000). One of the strongest ways in which the need for relatedness is fulfilled is by feeling understood (Reis et al., 2000), or the sense that another person “gets you” in some fundamental way (Reis et al., 2017). The need for relatedness is particularly frustrated during loss: Not only is bereavement a direct loss of a close and connected other, but when people return to work after losing a loved one, they often encounter organizational norms that prioritize positivity and professionalism over vulnerability (Barros-Lane et al., 2014). This mismatch may lead their grief to become “disenfranchised,” where their grief is minimized through implicit and explicit messages that discourage griever from talking about the true extent of their pain (Barros-Lane et al., 2014)

which stunts their ability to heal from their loss (Stroebe & Schut, 2010) and may dissociate them from others (Srivastava et al., 2009). Furthermore, a core proposition of Basic Psychological Needs Theory is that when needs are “frustrated” or left unmet, people will engage in attempts to satisfy this unmet need (Ryan & Deci, 2017). Indeed, this is precisely what grievers do: When asked what their organizations can do to better support employees suffering from grief, employees report that the recognition and acknowledgment of their loss, as well as being shown care from others, are paramount (Gilbert et al., 2021), and when grievers return to work after losing a loved one or after a romantic breakup, they deepen and broaden their interactions with their colleagues (Pletneva, 2024).

Although Basic Psychological Needs Theory is often tested at the individual level, one of the core reasons why validation will fulfill one’s need for relatedness is because it is more likely to make grievers feel understood (Reis et al., 2017). Feeling understood refers to the griever’s belief that the sympathizer “gets them” in some fundamental way. By accurately labelling and reflecting on an experience (e.g., “That sounds like it is really tough”), validation creates a shared reality – a perceived commonality of inner states (Echterhoff et al., 2017) – that reduces the sense that one is alone in their experience. Validation also legitimizes one’s experience (e.g., “It makes sense to feel this way, you two were really close”), and in so doing, reduces the sense that one will be judged or disconnected from others in their grief (Barros-Lane et al., 2014). And by accepting another’s inner world, messages of validation are more likely to facilitate the perception that one can be authentic with the sympathizer. That is, when grievers feel that their inner world will not be judged, corrected, or minimized, grievers will be more likely to experience the interaction as one in which they can “be themselves” (Grandey & Melloy, 2017). For these reasons, I hypothesize:

Hypothesis 3a: Validation is more effective at alleviating distress than positive reframing.

Hypothesis 3b: Feeling understood mediates the effect of validation (versus positive reframing) on distress alleviation.

But will validation always outperform positive reframing? According to my argument thus far, validation will be more effective than positive reframing because validation is more likely to satisfy one's need for relatedness, primarily by making grievers feel understood. However, different losses thwart different psychological needs. For example, bereavement often threatens relatedness by severing a core bond and intensifying feelings of isolation (Stroebe & Schut, 2010), whereas an event such as job loss often threatens self-worth and competence by calling one's value and capability into question (Prause & Dooley, 1997). This implies something important: positive reframing backfires not because it is largely ineffective, but because it is misaligned with the psychological need that is thwarted in bereavement.

The need for esteem is the need to experience oneself as a person of value (i.e., competent, worthy, and deserving of respect) rather than inadequate or inferior (Dweck, 2017). I argue that positive reframing is likely to be particularly effective in loss contexts in which one's need for esteem and/or competence is frustrated, such as job loss, a failed promotion, reputational damage, and divorce. Positive reframing frequently emphasizes a future trajectory (e.g., improvement, growth, or benefit finding), which restores confidence in one's perceived capability to recover (Helgeson et al., 2006). And, when communicated by another person, positive reframing can restore seeing oneself as someone who is valued by others despite the adversity (Lille et al., 2018). Accordingly, I predict that the relative effectiveness of validation versus positive reframing depends on which need is thwarted by the loss event:

Hypothesis 4a: When one's need for esteem is thwarted, positive reframing is more effective than validation because positive reframing addresses esteem needs.

Hypothesis 4b: When one's need for relatedness is thwarted, validation is more effective than positive reframing because validation addresses relatedness needs.

Overview of Studies

In Study 1, an archival analysis of over 600 sympathy cards, I find that positive reframing is substantially more prevalent than expressions of validation (Hypothesis 1). In Study 2a, I examined the prevalence of positive reframing in expressions of sympathy and show how greater empathic concern (Hypothesis 2) predicts the prevalence of expressing a positive reframe over validation. In Study 2b, I show that medical professionals, who frequently have to sympathize with others, are less likely to make this error, especially when they have been trained in bad news delivery. In Study 3a and Study 3b, I shift the focus to the griever's perspective and find that validation is more effective than positive reframing (Hypothesis 3a), which is mediated by increased feelings of being understood (Study 3b). Lastly, in Study 4, I show that the psychological need that is thwarted from the loss moderates the effectiveness of the sympathy expression, such that positive reframing is more effective when esteem is thwarted (Hypothesis 4a) and validation is more effective when relatedness is thwarted (Hypothesis 4b).

Transparency and Openness

In all studies, the target sample size was determined prior to data collection based on a sensitivity power analysis. I report all data exclusions (if any), all manipulations, and all measures in every study (Simmons et al., 2011). The data, code, and materials for all studies, as well as the preregistrations are accessible on our Open Science Framework (OSF) repository (https://osf.io/zwe9v/overview?view_only=ba58b7da9b14472bad901aa954a6901b)

Study 1: Sympathy Cards

To examine Hypothesis 1, that positive reframing is more prevalent than expressions of validation, I purchased all sympathy cards from stores in five locations in North America (see Method) and coded the content of the cards. I focused on the content in sympathy cards because they are cultural products in that they are widely distributed objects that both reflect and reinforce cultural norms and ideas, and thus are a good means to assess dominant ideas and practices that shape how people respond to loss and grief (e.g., Koopmann-Holm & Tsai, 2014).

Method

Content Coding. To select a representative sample of sympathy cards, I collected and purchased all sympathy, bereavement, and related (e.g., illness) cards in five different locations in North America (i.e., Atlanta, Boston, Dallas, San Francisco, Toronto; $N = 632$ cards). Given that Large Language Models (LLMs) like ChatGPT can detect psychological constructs with greater accuracy than standard dictionary-based methods – and yield ratings highly correlated with human judgments (Rathje et al., 2024) – I trained ChatGPT4o to code for the presence (1) or absence (0) of validation (i.e., expressing an understanding of the griever’s experience without directly trying to change it) and positive reframing (i.e., shifting the griever’s perspective to focus on the potential positive aspects, benefits, or outcomes of their loss). I then gave the same instructions to a research assistant, who was blind to the research question, and asked them to complete the same coding task. The inter-rater reliability between ChatGPT4o and the research assistant was acceptable for both positive reframing, $\kappa = .76$ (88% agreement) and validation, $\kappa = .70$ (97% agreement).

Results and Discussion

Supporting Hypothesis 1, sympathy expressions that positively reframed one's loss (e.g., "What we once enjoyed and deeply loved we can never lose for all that we love deeply become a part of us") were significantly more prevalent (35.1%, $n = 222$) than expressions that validated one's experiences (5.4%, $n = 34$; e.g., "This really truly sucks. I wish you didn't have to go through this"), $\chi^2(1) = 173.13, p < .001$. In fact, positive reframing was 8.67 times more prevalent, 95% CI [5.92, 12.71] than validation. Positive reframing and validation were not significantly associated with each other, $\chi^2(1) = 0.94, p = .360, OR = 0.65, 95\% CI [0.28, 1.39]$. Therefore, when expressing sympathy, there may be a tendency for sympathizers to try to change or regulate the griever's emotional experience, and a common way of doing so is to point towards the 'silver lining' in their loss.

Study 2a: Predictors of Sympathy Expressions

In preregistered Study 2a (<https://aspredicted.org/jfzn-ssdz.pdf>), I shift the focus to lay expressions of sympathy to assess whether positive reframing is indeed more common than validation when naturally consoling someone through their grief. I also test Hypothesis 2, examining whether those who are most empathically concerned will be most likely to express a positive reframe over validation.

Method

Participants. Participants were 1,000 adults ($M_{\text{age}} = 40.4, SD_{\text{age}} = 13.6$; 56.0% female, 1.0% Non-binary/other) recruited from Prolific.

Procedure. Participants first watched a video of a target describing the "worst day of her life," which was the day her father passed away. After watching the video, participants then completed measures of empathic concern and empathic distress (Batson et al., 1987). Then, they

were provided with a text box and were asked, “What would you say to the person in the video?”. Participants were told to “Please keep in mind that your message may be sent to someone who has recently experienced a similar type of loss.” Lastly, participants completed a few demographic items (i.e., age, gender, race), as well as their own previous experience with loss.

Measures

Strategy. To detect positive reframing and validation, I leveraged a psychometric natural language processing approach (see Mousavi et al., 2023) where I designed, fine-tuned, and applied an advanced Natural Language Processing (NLP) model, BERT. I chose to train and fine-tune BERT models because previous research has shown that they are more accurate at detecting relevant constructs (e.g., resilience, empathy) compared to dictionary-based methods as well as other custom-build models (Alavi et al., 2024). In training and evaluating the models on a separate data set,¹ the results revealed that the positive reframing NLP model correctly classified 97.7% of the messages and the validation NLP model correctly classified 99.5% of the messages. I therefore applied these two NLP models to the current data.

State Empathic Distress and State Empathic Concern (Batson et al., 1987). Prior to writing their message, participants were asked on a scale from 1 (*not at all*) to 7 (*very much so*) anchored at 4 (*somewhat*) to what extent they felt empathically distressed (8-items, e.g., “distressed;” $\omega = .94$), and empathically concerned (6-items, e.g., “sympathetic;” $\omega = .90$) as a result of what the person in the video described.

Participants also completed other exploratory items, and demographic measures that are available on OSF.

¹ Due to space limitations, I document the development of the NLP models here: <https://tinyurl.com/4dfezeyd>.

Results

Prevalence of Positive Reframing vs. Validation. Supporting Hypothesis 1, positive reframing was significantly more prevalent (24.4%, $n = 243$) than expressing validation (13.6%, $n = 136$), $\chi^2(1) = 36.60$, $p < .001$. In fact, sympathizers were 2.04 times more likely, 95% CI [1.62, 2.57], to express a positive reframe than they were to express validation. Positive reframing and validation were not significantly correlated with each other, $\chi^2(1) = 0.87$, $p = .349$, $OR = 1.24$, 95% CI [0.83, 1.86].

Predictors of Positive Reframing vs. Validation. To examine the if empathic concern (Hypothesis 2) predicts a greater probability of expressing a positive reframe over validation, I conducted a multinomial regression model predicting expression type by empathic concern, empathic distress, and previous experience with loss. I re-coded the sympathy expression variable to predict whether the sympathizer expressed a positive reframe (20.6%, $n = 205$), validation (9.8%, $n = 98$), a mixed positive reframe/validating message (3.8%, $n = 38$), or neither (65.8%, $n = 656$). Considering my main hypotheses concern the probability of expressing a positive reframe over validation, I focus my analyses on this specific contrast (but see Supplementary Study 2a on the OSF Repository for all contrasts).

Supporting Hypothesis 2, empathic concern significantly predicted the probability of expressing a positive reframe over validation, $b = 0.30$, $SE = 0.11$, $p = .005$. That is, a one-point increase in empathic concern predicted 35% increased odds, $OR = 1.35$, 95% CI [1.09, 1.66], of expressing a positive reframe over validation. Empathic distress did not significantly predict the probability of expressing a positive reframe over validation, $b = -0.09$, $SE = 0.09$, $p = .303$, $OR = 0.92$, 95% CI [0.77, 1.08]. Importantly, all results held after controlling for the length of the message.

Study 2b: What Do People Who Regularly Express Sympathy?

Study 2a raises a critical question: Does this pattern generalize to contexts where sympathizing is a regular job requirement? In Study 2b, I examined sympathy expressions in a sample of individuals who must frequently sympathize with others: medical professionals (e.g., Baile et al., 2000). Specifically, I asked medical professionals to communicate a message to a person who was grieving, and I compared their responses to a separate sample of laypersons. This study was preregistered (<https://aspredicted.org/hc8w7y.pdf>).

Method

Participants. Medical professionals consisted of 155 individuals (78.1% physicians, 1.3% registered nurses, and 20.6% ‘Other’, such as medical educators and professors) who had an average of 18.7 years of experience ($SD = 10.8$, range = 0-58) on the job. Medical professionals were recruited from medical listservs (i.e., PED-EM-L and HIFA), snowball sampling, and from personal contacts. The majority of participants were women (68.5%, $n = 89$; $M_{age} = 47.0$, $SD_{age} = 11.2$, range = 30-85) who were employed in the United States (93.8%, $n = 122$). A comparison layperson sample consisting of 150 adults (48.7% women; $M_{age} = 38.8$, $SD_{age} = 11.9$, range = 21-73) were recruited from Prolific.

Procedure. Medical professionals were asked to indicate how they would respond to the distressed daughter of a patient who passed away the previous night after a battle with pancreatic cancer. Laypersons were given an equivalent scenario but imagined that the target was a coworker. For both samples, participants were provided with a text box and were asked, “What do you say in response? Please respond as you naturally would in this moment, word-for-word.” Lastly, participants completed a few demographic items (i.e., age, gender, race, country of residence), and medical professionals completed a series of other medical questions (see below).

Measures

Strategy. To detect positive reframing and validation, two people coded for whether the message expressed positive reframing or validation. Any discrepancies were reconciled by a third coder.

Exploratory Medical Questions. Medical professionals were asked a series of additional exploratory questions, such as the number of years they worked in medicine (open ended), their speciality (open-ended), and if they have received formal training in how to deliver bad news to patients or families (1 = *yes*, 2 = *no*, 3 = *unsure*), and if *yes* what type of training they received (e.g., simulated patient scenarios/role-play). All items are available via OSF.

Demographics. Participants also reported their age, the gender they identify with (Man, Woman, Non-Binary/Third gender, Prefer not to say), their race/ethnicity (White, Black, Hispanic, Asian, Native American, Pacific Islander, Mixed or Other) and their country of residence (US, Canada, or Other).

Results

Prevalence of Strategy. To examine if the difference in prevalence for positive reframing versus validation is greater for medical professionals versus laypersons, I conducted a multinomial regression model predicting strategy, separately by sympathizer (Medical Professionals vs. Laypersons). I re-coded the strategy variable to predict whether the sympathizer expressed a positive reframe (13.9%, $n = 39$), validation (19.6%, $n = 55$), a mixed positive reframe/validating message (4.6%, $n = 13$), or neither (60.5%, $n = 170$). Considering my main hypotheses concern the probability of expressing validation over a positive reframe, I focus my analyses on this specific contrast (but see Supplementary Study 2b on the OSF Repository for all contrasts). Results suggested that medical professionals were significantly more likely to express

validation (26.4%, $n = 34$) compared to positive reframing (5.4%, $n = 7$), $b = 1.58$, $SE = 0.42$, $p < .001$, but laypersons were slightly more likely to express positive reframing (23.5%, $n = 32$) instead of validation (14.7%, $n = 20$), $b = 0.47$, $SE = 0.29$, $p = .099$. Put another way, medical professionals were 4.86 times more likely, 95% CI [5.55, 25.94], to express validation as opposed to positive reframing, whereas laypersons were 1.60 times more likely, 95% CI [2.58, 6.84] to express positive reframing over validation

To explore why medical experts are more likely to express validation compared to positive reframing, I conducted a series of exploratory multinomial regression models predicting strategy. Only having received training on delivering bad news marginally predicted this difference, $b = 1.64$, $SE = 0.87$, $z = 1.88$, $p = .061$. In other words, medical professionals who report having received training in bad news delivery had 5.14 higher odds, 95% CI [0.55, 12.52], of communicating validation rather than a positive reframe, compared to medical professionals with no/uncertain training.²

Study 2 Discussion

In Study 2a, participants expressed sympathy to a target who was grieving the loss of her father. Supporting Hypothesis 1, sympathizers were significantly more likely to encourage the griever to ‘look on the bright side’ than they were to validate their experiences. The data further suggest that this was not a result of being cold or uncaring; instead, those who felt the most compassion were the most likely to express a positive reframe over validation (Hypothesis 2). In

² Medical professionals indicated a number of different types of received training, including simulated scenarios/role-play (66.7%, $n = 58$), residency/clinical training (58.6%, $n = 51$), shadowing/observing experienced physicians (52.9%, $n = 46$); workshop or seminar (41.4%, $n = 36$), medical coursework (35.6%, $n = 31$), online module or video training (19.5%, $n = 17$), and/or other (e.g., teaching, research; 10.3%, $n = 9$). Importantly, the specific type of training did not significantly predict the probability of expressing validation over a positive reframe, $ps \geq .287$.

other words, expressing a positive reframe appears to be motivated by an altruistic goal to alleviate another's suffering.

In Study 2b, I compared how medical professionals versus laypersons express sympathy to a person grieving the loss of her father. Results indicated that medical professionals showed a very different pattern of results, such that they were substantially more likely to express validation over positive reframing. Although my analysis was exploratory and underpowered, I found that having received training on bad news delivery seemed to matter. In other words, one reason that expressing validation is so relatively infrequent compared to positive reframing for laypersons may be due to the fact that it is difficult to do and needs to be explicitly taught.

Study 3a: Active Grievers

To examine which strategy is more effective at alleviating suffering, in preregistered Study 3a (<https://aspredicted.org/wmkh-g8pf.pdf>) I shifted to the griever's perspective and randomly assigned them to receive and evaluate a message that either expressed a positive reframe or validation.

Method

Participants. Participants were 496 adults ($M_{\text{age}} = 40.17$, $SD_{\text{age}} = 12.32$, 55.0% female, 1.6% Other/Non-binary) who had experienced the loss of a close loved one in the past year. A target sample size of 500 was pre-determined because it gave me 95% power (with $\alpha = .05$ and a two-tailed test) to observe a small difference between two independent means ($d = 0.32$).

Procedure. First, I screened for participants who indicated that they experienced the death of a close family member in the past year ($N = 2500$), and if they indicated "Yes" ($n = 942$), they were eligible to participate in the experiment. After consenting to participate, grievors ($N = 496$) were asked to "Please recall the time in the past year when a close family member

passed away” and they were encouraged to “Take a few moments to reflect on how you felt in this situation”. They were then randomly assigned to receive a message that either expressed a positive reframe or validation (see Study 3a on the OSF Repository for the messages participants received). For instance, one validating message was:

“I can’t imagine how difficult this must be for you, and it’s completely understandable to feel a wide range of emotions right now. Losing someone you love sounds incredibly hard, and however you’re processing it—whether you’re feeling overwhelmed, numb, or anything in between—makes total sense.”

One example of a positive reframe read:

“I truly believe this loss will help you cherish the relationships and experiences that are still ahead. The person you lost would want you to live fully, learn from this, and find new joy and meaning as you move forward.”

To generalize my findings to other non-sampled stimuli (Yarkoni, 2022), I took a stimulus sampling approach whereby grieverers were randomly assigned to receive 1 of 5 messages that expressed a positive reframe or 1 of 5 messages that expressed validation. After reading the message, participants completed the survey measures (see below). Lastly, grieverers completed manipulation checks and a few demographic questions.

Measures

All variables were measured on a scale from 1 (*not at all*) to 7 (*very much*) anchored at 4 (*somewhat*).

Effectiveness. After receiving the message, grieverers were asked, “In the moment, how effective would this be in making you feel better?” Additionally, they were also asked to explain their response in an open-ended text box after selecting their response. Grieverers were also asked, “In the moment, to what extent would this make you feel worse?”

Empathic Concern. I asked grieverers to indicate how empathically concerned they believed the supposed sympathizer to be. Empathic concern was measured via six items: “To

what extent would you believe this person felt ... Sympathetic; Moved; Tender; Compassionate; Warm; and Soft-hearted” ($\omega = 0.95$; Coke et al., 1978).

Future Disclosure. I measured griever’s willingness to disclose a personal negative event in the future with one item: “How likely would you be to share a personal negative experience (such as death, illness, or job loss) with this person in the future?”.

Manipulation Checks. Participants completed two manipulation check items: one for validation (“To what extent did the message focus on making you feel understood, without suggesting ways to fix or change your emotions?”) and one for positive reframing (“To what extent did the message focus on the potential positive aspects, benefits, or outcomes of your loss?”).

Demographics. Finally, griever’s were asked to indicate their age, gender, time (in months) since their loved one passed away, and how close their relationship was with the deceased on a scale from 1 (*not at all*) to 7 (*very much*) anchored at 4 (*somewhat*).

Results

Due to the multilevel nature of the data, whereby messages were nested within expression type (i.e., positive reframing and validation), I ran a series of mixed OLS regression models predicting the dependent variables by the type of expression (dummy coded such that 0 = positive reframing, 1 = validation), with the specific expressions (i.e., the five positive reframing messages and the five validating messages) clustered within condition as random effects.

Manipulation Checks. Results of the manipulation checks indicated that griever’s who received a validating message rated the message as significantly more validating ($M = 5.30$, $SE = 0.13$) than those who received a message that positively reframed their loss ($M = 4.06$, $SE = 0.13$), $t(8.13) = 6.61$, $p < .001$, $d = 0.79$, 95% [0.51, 1.08]. Conversely, griever’s who received a

message that positively reframed their loss were significantly more likely to report that the message focused on the positive aspects, benefits, or outcomes of their loss ($M = 5.06$, $SE = 0.11$) compared to those who received a validating message ($M = 4.04$, $SE = 0.11$), $t(8.13) = 6.50$, $p < .001$, $d = 0.63$, 95% CI [0.41, 0.86].

Effectiveness. Results of the multilevel model suggested that there was a significant effect of expression type on how effective the message would be in making grievors feel, such that expressions of validation would be more effective ($M = 4.65$, $SE = 0.11$) than expressions that positively reframe one's loss ($M = 3.98$, $SE = 0.11$), $t(8.21) = 4.31$, $p = .002$, $d = 0.42$, 95% CI [0.19, 0.65]. Similarly, there was a significant effect of expression type on the extent to which the expression would be ineffective, such that expressions of validation would be less likely to make grievors feel worse ($M = 2.31$, $SE = 0.13$) compared to expressions that positively reframe one's loss ($M = 2.97$, $SE = 0.13$), $t(7.94) = -3.56$, $p = .007$, $d = -0.40$, 95% CI [-0.66, -0.14]. Therefore, despite positive reframing being a much more frequent strategy for expressing sympathy, expressions of validation are more effective at alleviating (and not contributing to) the suffering of grievors.

Empathic Concern. I also found a significant effect of expression type on how empathically concerned grievors would perceive sympathizers to be. Specifically, grievors would perceive those who express a validating message to be more empathically concerned ($M = 5.27$, $SE = 0.10$) than those who express a positive reframe ($M = 4.83$, $SE = 0.10$), $t(8.12) = 3.07$, $p = .015$, $d = 0.31$, 95% CI [0.08, 0.55]. That is, despite empathic concern being a positive predictor of expressing a positive reframe over validation, grievors perceive those who express validation to be more empathically concerned.

Future Disclosure. Results indicated that grievors who received a validating message were significantly more willing to disclose a negative personal event in the future to the sympathizer who expressed a validating message ($M = 4.74$, $SE = 0.11$) compared to those who expressed a positive reframe ($M = 4.06$, $SE = 0.11$), $t(8.21) = 4.15$, $p = .003$, $d = 0.40$, 95% CI [0.18, 0.62].

Study 3b: The Importance of Feeling Understood

To replicate and extend the effects of Study 3a, in preregistered Study 3b (<https://aspredicted.org/m49ww8.pdf>), I randomly assigned active grievors to one of four conditions in a 2 (Message: Positive Reframing vs. Validation) X 2 (Sympathizer: Doctor vs. Coworker) between-subjects design. Because the scenario was matched to Study 2b, participants were also randomized to sympathizer types (doctor vs. coworker), though I had no a priori predictions about sympathizer type. I also tested Hypothesis 4b by measuring the extent to which the messages made participants feel understood.

Method

Participants. Participants were 1,000 adults ($M_{age} = 40.9$, $SD_{age} = 12.5$, 53.1% female) who had experienced the loss of a close loved one in the past twelve months. A target sample size of 1,000 was pre-determined because it gave me 95% power (with $\alpha = .05$ and a two-tailed test) to observe a small difference between two independent means ($d = 0.23$).

Procedure. First, I screened for participants who indicated that they experienced the death of a close family member in the past year ($N = 4,000$), and if they indicated “Yes” ($n = 1,413$), they were eligible to participate in the experiment. After consenting to participate, active grievors ($N = 1,000$) were asked to imagine the scenario from Study 2b in which their loved one battling pancreatic cancer had passed away. They were then randomly assigned to receive a

message (taken from Study 3a) that either expressed a positive reframe or validation from either a coworker or a doctor (see Study 3b on the OSF repository for the messages participants received). For instance, one validating message was: “I’m sorry for your loss. I know that this must feel so difficult to hear, especially having seen how close you were with your loved one”. One example of a positive reframe read: “I’m sorry for your loss. One silver lining is that you got a lot of time with them, so you’re very lucky to have as many memories as you do.”

Similar to Study 3a, I again took a stimulus sampling approach whereby grievors were randomly assigned to receive 1 of 5 messages that expressed a positive reframe or 1 of 5 messages that expressed validation. Participants then completed the measures of effectiveness, ineffectiveness, and empathic concern from Study 3a. To measure feeling understood (Reis et al., 2017), I asked active grievors, “To what extent would this message make you feel...Like someone understands what you’re going through; Seen or heard; Alone in your difficulty (reversed; $\omega = 0.82$)”. I also measured grievors’ preferences for future support with one item: “To what extent would you want this person to comfort you during a future difficult experience?” Participants also completed demographic questions available via OSF.

Results

First, again finding support for Hypothesis 3a, expressions of validation were more effective ($M = 4.18$, $SE = 0.13$) than expressions that positively reframe one’s loss ($M = 3.29$, $SE = 0.13$), $t(14.00) = 4.32$, $p < .001$, $d = 0.54$, 95% CI [0.29, 0.79]. I also found a significant main effect of expression type on empathic concern, such that grievors perceived those who expressed a validating message to be more empathically concerned ($M = 5.24$, $SE = 0.19$) than those who expressed a positive reframe ($M = 4.49$, $SE = 0.19$), $t(9.77) = 3.00$, $p = .014$, $d = 0.56$, 95% CI [0.12, 0.99]. Lastly, grievors were significantly more willing to turn to the sympathizer for future

support when they expressed validation, ($M = 4.42$, $SE = 0.17$) compared to a positive reframe ($M = 3.53$, $SE = 0.17$), $t(11.70) = 3.84$, $p = .002$, $d = 0.52$, 95% CI [0.22, 0.83]. There were no significant main effects of sympathizer, $ps \geq .270$, and no significant interactions between message and sympathizer, $ps \geq .230$. That is, despite empathic concern being a positive predictor of expressing a positive reframe over validation, grievors perceive those who express validation to be more empathically concerned.

Next, to examine if feeling understood (i.e., that the sympathizer “gets” what the griever is going through) was the mechanism that could explain the beneficial effects of validation over positive reframing, I ran several percentile bootstrap mediation models with 10,000 resamples using the *robmed* package in *R* (Alfons et al., 2022). Supporting Hypothesis 3b, results suggested that validation was indirectly related to greater effectiveness, $b = 0.84$, 95% CI [0.69, 0.99], greater perceived empathic concern, $b = 0.76$, 95% CI [0.63, 0.90], and a greater willingness to turn to that person for support in the future in the future, $b = 0.90$, 95% CI [0.74, 1.06], through greater levels of feeling understood. Therefore, regardless of whether a griever receives a validating message from a doctor or a coworker, one of the potential reasons why expressing validation is a more effective message than expressing a positive reframe during bereavement is because such expressions are more likely to make one feel understood.

Study 3 Discussion

Shifting to the griever’s perspective in Study 3³, I found support for Hypothesis 3a: positive reframing (versus validation) was less effective, those who express positive reframing (versus validation) were perceived to be less empathically concerned, and those who receive a

³ I have also replicated this finding using a critical incidence technique (Flanagan, 1954) whereby full-time employees recalled a positive reframe or a validating message from their manager during a range of loss events. Due to space limitations, I included this in the online supplement (see Supplemental Study 1 on the OSF repository).

positive reframe would be less willing to turn to that person for support in the future. Why might this be the case? Supporting Hypothesis 3b, in Study 3b I found that one of the strongest ways of satisfying one's need for relatedness – feeling understood (Reis et al., 2000) – mediated all observed effects. In other words, one of the core reasons why validation may be more effective during bereavement is that it is most likely to satisfy one's thwarted psychological need for relatedness.

Study 4: Moderation by Psychological Need

The purpose of Study 4 was to causally test Hypotheses 4a and 4b – that positive reframing is misaligned with the psychological need that is thwarted from bereavement. I focused on relational loss (divorce) in this study for two reasons. First, people evaluate a breakup or divorce as among one of the greatest stressors in life (Pletneva, 2024). And second, preliminary tests indicated that divorce was a loss event that participants perceived to thwart both relatedness and esteem needs (See Supplemental Study 4 on the OSF repository). My theory thus far suggests that when one's esteem need is made salient, positive reframing will be more effective, and this is because positive reframing will be more likely to make grievers feel good about themselves. Conversely, when one's relatedness need is made salient, validation will be more effective than positive reframing, and this is because validation will be more likely to make grievers feel understood. However, when both needs (or no needs) are made salient, there will be no differences between positive reframing and validation, because both strategies would satisfy different psychological needs, thereby attenuating any main effects. This experiment was preregistered (<https://aspredicted.org/fn9xu7.pdf>).

Method

Participants. Participants were 500 adults ($M_{\text{age}} = 41.2$, $SD_{\text{age}} = 12.8$, 51.8% female) recruited from Prolific. A target sample size of 500 was pre-determined because it gave me 95% power (with $\alpha = .05$ and a two-tailed test) to observe a small difference between two dependent means ($d = 0.32$).

Procedure. Participants were randomly assigned to one of four conditions in a 4 (Need: Esteem vs. Relatedness vs. Esteem + Relatedness vs. Control) X 2 (Message: Positive Reframing vs. Validation) mixed-subjects design, with Need manipulated between-subjects and Message manipulated within-subjects. Those in the Esteem condition were told:

Imagine you have recently gotten divorced. Since the divorce, you've started to see yourself as less capable and less worthy. You doubt your judgment, question your ability to make good choices, and wonder if you have what it takes to succeed in important areas of life. Your sense of self-worth and competence has taken a serious hit.

Those in the Relatedness condition were told:

Imagine you have recently gotten divorced. Since the divorce, you've felt a deep lack of connection and companionship. You feel like no one really understands you or what you're going through. Friends check in, but conversations quickly move on to lighter topics. Even when you're with others, you feel unseen and unheard, like your feelings are invisible.

Those in the Esteem + Relatedness condition were told:

Imagine you have recently gotten divorced. Since the divorce, you've started to see yourself as less capable and less worthy. Your sense of self-worth and competence has taken a serious hit. You've also felt a deep lack of connection and companionship. Even when you're with others, you feel unseen and unheard, like your feelings are invisible.

And those in the Control condition were told: *Imagine you have recently gotten divorced.*

Within each Need condition, participants evaluated both a validating message and a positive reframe, using language similar to previous studies.

Each message was rated in terms of the key measures: perceived effectiveness and willingness to share difficulty in the future as measured in Study 3b. Feeling understood was measured as in Study 3b, and esteem needs were measured with three items: “To what extent would this message make you feel...Confident in yourself, good about yourself, Like a failure (reversed)” ($\omega_{\text{Validation}} = .81$; $\omega_{\text{Reframing}} = .85$). Finally, participants completed demographics (age and gender), their marital status, whether they have been divorced, and if so how long ago it happened.

Results

To analyze the data, I first ran a series of repeated measured ANOVAs on effectiveness and willingness to share difficulty in the future by the fixed effect of Need, the within effect of Message, as well as their interaction. To further probe the mechanism that drives the moderated effects, I ran a series of preregistered bootstrap mediation models with 10,000 resamples within each Need condition. In each model, the IV was message type (i.e., Validation vs. Positive Reframing), the DV was the difference in effectiveness or future sharing (e.g., $\text{Effectiveness}_{\text{Validation}} - \text{Effectiveness}_{\text{Reframing}}$), mediator 1 was the difference in feeling understood (i.e., $\text{Understood}_{\text{Validation}} - \text{Understood}_{\text{Reframing}}$), and mediator 2 was the difference in feeling good about oneself (i.e., $\text{Good}_{\text{Validation}} - \text{Good}_{\text{Reframing}}$). Due to the repeated nature of the data, this allowed me to test if – for the same individual – positive reframing was more effective than validation because it was more likely to make them feel good about themselves or because it was more likely to make them feel understood (and vice versa).

Effectiveness. Results revealed no main effect of Need, $F(3, 496) = 1.37, p = .250, \eta^2 = .005$, no main effect of Message, $F(3, 496) = 0.05, p = .823, \eta^2 < .001$, but a significant Need X Message Interaction, $F(3, 496) = 8.10, p < .001, \eta^2 = .019$. In the Esteem Need condition,

results suggested that positive reframing was rated as significantly more effective than validation, $b = 0.73$, $SE = 0.19$, $p < .001$, and this is because positive reframing was more likely to make grievors feel good about themselves, $b = 0.35$, $SE = 0.12$, 95% CI [0.15, 0.61] as opposed to feeling understood, $b = 0.06$, $SE = 0.09$, 95% CI [0.13, -0.24]. On the contrary, in the Relatedness Need condition, despite validation only being marginally more effective than positive reframing, $b = 0.29$, $SE = 0.17$, $p = .091$, validation was indirectly related to greater effectiveness through a greater sense of feeling understood, $b = 0.37$, $SE = 0.11$, 95% CI [0.16, 0.61], as opposed to feeling good about oneself, $b = -0.34$, $SE = 0.06$, 95% CI [-0.15, 0.07]. Contrary to expectations, when both needs were made salient, validation was rated as significantly more effective than positive reframing, $b = 0.38$, $SE = 0.18$, $p = .028$, particularly because validation was more likely to make grievors feel understood, $b = 0.22$, $SE = 0.10$, 95% CI [0.06, 0.45] as opposed to feeling good about themselves, $b = -0.09$, $SE = 0.09$, 95% CI [-0.28, 0.06]. And lastly, as expected, in the Control condition, there were no significant differences in the effectiveness of message type, $b = 0.14$, $SE = 0.18$, $p = .451$, and this is because validation was more likely to make grievors feel understood, $b = 0.19$, $SE = 0.09$, 95% CI [0.04, 0.38], and positive reframing was more likely to make grievors feel good about themselves, $b = 0.27$, $SE = 0.09$, 95% CI [0.10, 0.47]. In other words, positive reframing is especially more effective than validation when one's need for esteem is thwarted, and validation is more effective when one's need for relatedness is thwarted.

Sharing. Results for being willing to share difficulty with the sympathizer in the future revealed no main effect of Need, $F(3, 496) = 0.13$, $p = .943$, $\eta^2 < .01$, a significant main effect of Message, $F(3, 496) = 5.19$, $p = .023$, $\eta^2 = .004$, and a significant Need X Message Interaction, $F(3, 496) = 5.23$, $p = .001$, $\eta^2 = .013$. Follow-up mediation analyses revealed that,

when validated, grievors are more willing to share difficulty in the future, but only when their need for Relatedness is made salient. That is, in the Relatedness Need condition, grievors would be more willing to share difficulty in the future when their experiences were validated as opposed to reframed, $b = 0.46$, $SE = 0.15$, $p = .003$, because such messages were more likely to make them feel understood $b = 0.35$, $SE = 0.11$, 95% CI [0.16, 0.59], as opposed to good about themselves, $b = -0.02$, $SE = 0.04$, 95% CI [-0.09, 0.05]. Similarly, when Both Needs were thwarted, grievors would be more willing to share difficulty in the future when their experiences were validated as opposed to reframed, $b = 0.57$, $SE = 0.17$, $p = .001$, because such messages were more likely to make them feel understood $b = 0.33$, $SE = 0.12$, 95% CI [0.13, 0.58], as opposed to good about themselves, $b = -0.05$, $SE = 0.05$, 95% CI [-0.16, 0.03]. However, in the Control condition – where no needs were made particularly salient – grievors were not more willing to share difficulty in the future when their experiences were validated as opposed to reframed, $b = 0.14$, $SE = 0.18$, $p = .434$, primarily because validation was more likely to make grievors feel understood, $b = 0.31$, $SE = 0.13$, 95% CI [0.07, 0.59], but positive reframing was more likely to make grievors feel good about themselves, $b = 0.15$, $SE = 0.07$, 95% CI [0.03, 0.31]. In the Esteem Need condition, grievors were not more willing to share difficulty in the future when their experiences were reframed as opposed to validated, $b = 0.35$, $SE = 0.21$, $p = .101$, in which positive reframing was not indirectly related to a greater willingness to share by being more likely to make grievors feel good about themselves, $b = 0.20$, $SE = 0.13$, 95% CI [0.46, -0.04] or understood, $b = 0.10$, $SE = 0.15$, 95% CI [0.37, 0-.20]. In other words, when validated, grievors are more willing to share difficulty with the supposed sympathizer in the future, but only when their need for relatedness is thwarted.

Study 4 Discussion

Supporting Hypothesis 4a, I found that positive reframing was more effective than validation when one's need for esteem was thwarted, and this is because positive reframing was more likely to make griever feel good about themselves. In partial support of Hypothesis 4b, I found that validation was indirectly rated as more effective than positive reframing when one's need for relatedness was thwarted, and this is because validation was more likely to make griever feel understood.

General Discussion

If someone is grieving, what might you express to best support them? Supporting Hypothesis 1, if you were to go to your local Hallmark store and select a sympathy card at random, you would be eight times more likely to select a card that points to the 'silver lining' or 'bright side' of their loss as opposed to a card that validates their experiences (Study 1). And a similar fact is true when you express sympathy in your own words (Study 2a). Supporting Hypothesis 2, such an inclination to positively reframe their distress may be a well-intentioned behavior to alleviate their suffering, and not a consequence of feeling cold or uncaring (Study 2a). However, despite your benevolent intentions, positive reframing is more likely to backfire. Supporting Hypothesis 3a, when griever were randomly assigned a message that either validated vs. positively reframed their experiences, both from someone they know (Study 3a), or from a doctor versus a coworker (Study 3b), griever reported that the message was more effective in alleviating their distress, that sympathizers were more compassionate, and they would be more willing to disclose difficulty to them again in the future (Hypothesis 3a), and this is because validation was more likely to make griever feel understood (Hypotheses 3b).

But does positive reframing always backfire? Supporting Hypothesis 4a, when one's need for esteem was thwarted, grievors rated positive reframing as more effective than validation, and this is because positive reframing was more likely to make grievors feel good about themselves. On the contrary, supporting Hypothesis 4b, when one's need for relatedness was thwarted, validation was rated as more effective than positive reframing through an increased sense that one felt understood by another. In other words, supportive communication is most effective when it is aligned with a psychological need that is thwarted from the loss event.

Theoretical Contributions

This research makes several theoretical contributions to the literature. First, I contribute to compassion theory (Dutton et al., 2014) by unpacking the 'black box' of what people communicate as a form of compassionate action (Atkins & Parker, 2012). Previous research has predominately operationalized compassion as something that is present or absent (e.g., Grant et al., 2008) or offered or withheld (Batson et al., 1988). However, this binary framing of compassionate action overlooks the nuances by which people may respond compassionately. The current research expands compassion theory by integrating the interpersonal emotion regulation literature (Zaki & Williams, 2013) with the compassion literature (Dutton et al., 2014) to show that the most common ways of expressing sympathy – motivated by those who feel the most compassionate for someone in need – may be the most likely to backfire. In this way, I broaden the literature on compassion, opening new avenues of inquiry into compassion efficacy and the differential outcomes associated with various compassionate strategies.

I also contribute to the interpersonal emotion regulation literature (Zaki & Williams, 2013) by more explicitly bringing supportive communication into this space (Nozaki & Gross, 2025). Although previous research links empathic concern with affect-improving emotion

regulation goals (Nozaki & Gross, 2025), it is not clear what people explicitly communicate when trying to achieve those goals. In digging deeper into this process, I challenge an important assumption in the interpersonal emotion regulation literature: that errors from strategy selection “most likely stem from a lack of knowledge and thus most prominently reflect deficits in emotion-related cognitive abilities, and broader skills, such as person perception and social cognition” (Niven et al., 2024: 6). Instead, I show that errors in strategy selection may often come from those who care the most. I therefore expand the empathy-altruism hypothesis (Batson et al., 2024) by demonstrating the ways in which empathic concern may pose an additional interpersonal liability, particularly in the domain of supportive communication.

Third, I contribute to Basic Psychological Needs Theory by showing that need satisfaction is not only an intrapersonal outcome of one’s activities and environments, but also an interpersonal process enacted in moment-to-moment interaction. By integrating interpersonal emotion regulation – specifically, the communicative strategies people use to change versus validate others’ emotions – I demonstrate that supportive messages can function as a mechanism of need support. In particular, when a loss frustrates relatedness needs, validation communicates understanding and acceptance, thereby restoring connection; when a loss frustrates esteem/competence needs, positive reframing can restore self-worth by signaling confidence in the self. This need-strategy alignment framework moves need satisfaction “out of the head” within individuals and into the interaction between them, offering a concrete basis for predicting when supportive communication will backfire, when it will help, and how it can be improved.

Practical Implications

This research has several important practical implications. In providing supportive communication (e.g., in healthcare or enforcement contexts), organizational actors may

intuitively select those who feel the most compassion for a suffering target. However, in doing so, those selected may be the very individuals who express support in a way that does not have its intended effect. In high stakes emotional contexts, this has incredibly important consequences, including diffusing school shootings (Tracy & Huffman, 2017), decreasing turnover (Grant et al., 2008), facilitating trust (Yu et al., 2021), and navigating a difficult diagnosis (Zhang et al., 2024).

This then, raises an important question: What can organizations do to ensure that support is communicated effectively? In the current research, expressions of validation were incredibly effective at supporting people going through a difficult time, but they were also incredibly rare. It is possible that, because navigating difficult emotions at work often feels awkward and perhaps taboo (Petriglieri & Maitlis, 2019), people may simply not know how to communicate validation or feel confident in doing so. Indeed, the focus on medical professionals (Study 3a), who sympathize quite frequently in the workplace, showed that they were substantially more likely to express validation compared to their layperson counterparts, and the data suggested that this may be due to having received training on bad news delivery. Organizations may therefore fare well by offering training and workshops – similar to what medical residents in oncology learn (Baile et al., 2000) – on expressing validation so that people feel equipped to express it when the situation inevitably calls for it.

Limitations and Future Directions

One limitation of the present research is that there are perhaps an infinite number of ways or tactics by which people can express a strategy, particularly positive reframing (Petrova & Gross, 2015). The literature identifies some different types of positive reframes, such as a temporal focus, a positive focus, and perspective taking (Sahi et al., 2023a). Indeed, in a supplementary study (see supplementary study 2 on the OSF Repository), I found that, overall,

validation was more effective than positive reframing in the context of losing a loved one, but some expressions (e.g., perspective taking with the deceased) were more effective than others (e.g., minimizing the loss). Future research could dig deeper into the different ways positive reframing is expressed in the context of different loss events and examine if certain tactics may be more effective than others.

Secondly, the current research examined “one shot” expressions of sympathy, but these conversations might unfold differently in real time, with facial expressions, vocal tones, physical touch, as well as conversational turn taking potentially playing a pivotal role (Stellar & Keltner, 2014; Yeomans et al., 2023). Future research can examine how interactions unfold when sympathizers express a positive reframe vs. validation, and how other nonverbal elements of these conversations (e.g., tone, facial expression) influence psychological and relational outcomes.

And lastly, although I was underpowered, I found that medical professionals may be more likely to express validation when they are trained in engaging in bad news delivery. Future research could examine which specific types of intervention, and for which specific loss events, facilitate the effectiveness of sympathy expressions in the workplace.

Conclusion

People overwhelmingly prioritize positive reframing when responding to loss – often because they care – despite validation being more effective at alleviating a griever’s distress, primarily by making grievers feel that someone understands what they are going through. Crucially, positive reframing is not inherently wrong: it becomes the better strategy when the loss primarily threatens one’s need for esteem/competence, highlighting that supportive communication works best when it aligns with the receiver’s thwarted psychological need.

References

- Alavi, M. Leidner, D. E., & Mousavi, R. 2024. A knowledge management perspective of Generative Artificial Intelligence. *Journal of the Association for Information Systems*, 25: 1-12.
- Alfons, A., Ateş, N. Y., & Groenen, P. J. F. 2022. Robust mediation analysis: The R package robmed. *Journal of Statistical Software*, 103(13): 1–45.
- Atkins, P. W., & Parker, S. K. 2012. Understanding individual compassion in organizations: The role of appraisals and psychological flexibility. *Academy of Management Review*, 37(4): 524-546.
- Baile, W. F., Buckman, R., Lenzi, R., Glober, G., Beale, E. A., & Kudelka, A. P. 2000. SPIKES—a six-step protocol for delivering bad news: Application to the patient with cancer. *The Oncologist*, 5(4): 302-311.
- Barros-Lane, L., Alvarez-Rodriguez, V., Cornelius, V. E., & Smith, V. D. 2024. “Grief lasts longer than sympathy”: Qualitatively examining disenfranchised grief in young widowhood. *Journal of Loss and Trauma*, 30(2): 1-22.
- Batson, C. D. 1994. Why act for the public good? Four answers. *Personality and Social Psychology Bulletin*, 20(5): 603-610.
- Batson, C. D. 2023. *Empathic concern: What it is and why it's important*. Oxford, UK: Oxford University Press.
- Batson, C. D., Dyck, J. L., Brandt, J. R., Batson, J. G., Powell, A. L., McMaster, M. R., & Griffitt, C. 1988. Five studies testing two new egoistic alternatives to the empathy-altruism hypothesis. *Journal of Personality and Social Psychology*, 55: 52-77.
- Batson, C. D., Fultz, J., & Schoenrade, P. A. 1987. Distress and empathy: Two qualitatively distinct vicarious emotions with different motivational consequences. *Journal of Personality*, 55(1): 19-39.
- Batson, C. D., O'Quin, K., Fultz, J., Vanderplas, M., & Isen, A. M. 1983. Influence of self-reported distress and empathy on egoistic versus altruistic motivation to help. *Journal of Personality and Social Psychology*, 45(3): 706–718.
- Bento, R. F. 1994. When the show must go on: Disenfranchised grief in organizations. *Journal of Managerial Psychology*, 9(6): 35-44.
- Bradley, C. M., Greer, L. L., Trinh, E., & Sanchez-Burks, J. 2024. Responding to the emotions of others at work: A review and integrative theoretical framework for the effects of emotion-response strategies on work-related outcomes. *Academy of Management Annals*, 18: 3-43.
- Brotheridge, C. M., & Lee, R. T. 2003. Development and validation of the emotional labour scale. *Journal of Occupational and Organizational Psychology*, 76(3): 365-379.
- Burleson, B. R. 2009. Understanding the outcomes of supportive communication: A dual-process approach. *Journal of Social and Personal Relationships*, 26: 21-38.
- Calderwood, K. A., & Alberton, A. M. 2024. Consoling the bereaved: Exploring how sympathy cards influence what people say. *OMEGA-Journal of Death and Dying*, 88(4): 1572-1590.
- Coke, J. S., Batson, C. D., & McDavis, K. 1978. Empathic mediation of helping: A two-stage model. *Journal of Personality and Social Psychology*, 36(7): 752–766.
- Deci, E. L., Connell, J. P., & Ryan, R. M. 1989. Self-determination in a work organization. *Journal of Applied Psychology*, 74(4): 580-590.

- Diefendorff, J. M., & Richard, E. M. 2003. Antecedents and consequences of emotional display rule perceptions. *Journal of Applied Psychology*, 88(2): 284–294.
- Dutton, J. E., Workman, K. M., & Hardin, A. E. 2014. Compassion at work. *Annual Review of Organizational Psychology and Organizational Behavior*, 1(1): 277-304.
- Dweck, C. S. 2017. From needs to goals and representations: Foundations for a unified theory of motivation, personality, and development. *Psychological Review*, 124(6): 689-719.
- Echterhoff, G., Higgins, E. T., & Levine, J. M. 2009. Shared reality: Experiencing commonality with others' inner states about the world. *Perspectives on Psychological Science*, 4(5): 496-521.
- Fleck, C. 2025. *Validation: How the skill set that revolutionized psychology will transform your relationships, increase your influence, and change your life*. London, UK: Penguin Group.
- Ford, B. Q. 2025. The individual, relational and societal costs of striving to feel good. *Nature Reviews Psychology*: 1-12.
- Fitch, G. 1970. Effects of self-esteem, perceived performance, and choice on causal attributions. *Journal of Personality and Social Psychology*, 16(2): 311–315.
- Flanagan, J. C. 1954. The critical incident technique. *Psychological Bulletin*, 51(4): 327–358.
- Gilbert, S., Mullen, J., Kelloway, E. K., Dimoff, J., Teed, M., & McPhee, T. 2021. The CARE model of employee bereavement support. *Journal of Occupational Health Psychology*, 26(5): 405–420.
- Goodman, W. 2022. *Toxic positivity: Keeping it real in a world obsessed with being happy*. London, UK: Penguin Group.
- Grandey, A. A., & Melloy, R. C. 2017. The state of the heart: Emotional labor as emotion regulation reviewed and revised. *Journal of Occupational Health Psychology*, 22(3): 407–422.
- Grant, A. M., Dutton, J. E., & Rosso, B. D. 2008. Giving commitment: Employee support programs and the prosocial sensemaking process. *Academy of Management Journal*, 51(5): 898-918.
- Gross, J. J. 2015. Emotion regulation: Current status and future prospects. *Psychological Inquiry*, 26: 1-26.
- Helgeson, V. S., Reynolds, K. A., & Tomich, P. L. 2006. A meta-analytic review of benefit finding and growth. *Journal of Consulting and Clinical Psychology*, 74(5): 797-816.
- Horowitz, L. M., Krasnoperova, E. N., Tatar, D. G., Hansen, M. B., Person, E. A., Galvin, K. L., & Nelson, K. L. 2001. The way to console may depend on the goal: Experimental studies of social support. *Journal of Experimental Social Psychology*, 37(1): 49-61.
- Jurkiewicz, O., & Oveis, C. 2025. Helping you helps me: Beneficial effects of regulating others' emotions on well-being and physiological stress. *Emotion*, 25(2): 372–386.
- Jurkiewicz, O., McGarrigle, C. B., & Oveis, C. 2023. How to improve others' emotions: Reappraise and be responsive. *Affective Science*, 4(2): 233-247.
- Kamdar, D., McAllister, D. J., & Turban, D. B. 2006. "All in a day's work": How follower individual differences and justice perceptions predict OCB role definitions and behavior. *Journal of Applied Psychology*, 91(4): 841-855.
- Koopmann-Holm, B., & Tsai, J. L. 2014. Focusing on the negative: Cultural differences in expressions of sympathy. *Journal of Personality and Social Psychology*, 107(6): 1092-1115.

- Lieberman, M. D., Inagaki, T. K., Tabibnia, G., & Crockett, M. J. 2011. Subjective responses to emotional stimuli during labeling, reappraisal, and distraction. *Emotion*, 11(3): 468–480.
- Lillie, H. M., Venetis, M. K., & Chernichky-Karcher, S. M. 2018. “He would never let me just give up”: Communicatively constructing dyadic resilience in the experience of breast cancer. *Health Communication*, 33(12): 1516-1524.
- Lin, S. H. J., Poulton, E. C., Tu, M. H., & Xu, M. 2022. The consequences of empathic concern for the actors themselves: Understanding empathic concern through conservation of resources and work-home resources perspectives. *Journal of Applied Psychology*, 107(10): 1843-1863.
- Linehan, M. M. 1997. Validation and psychotherapy. In A. C. Bohart & L. S. Greenberg (Eds.), *Empathy reconsidered: New directions in psychotherapy* (pp. 353–392). Washington, DC: American Psychological Association.
- Matthews, M., Webb, T. L., & Sheppes, G. 2022. Do people choose the same strategies to regulate other people’s emotions as they choose to regulate their own?. *Emotion*, 22(8): 1723–1738.
- Maynard, D. W. 1997. The news delivery sequence: Bad news and good news in conversational interaction. *Research on Language and Social Interaction*, 30(2): 93-130.
- Mousavi, R., & Gu, B. 2024. Resilience messaging: The effect of governors’ social media communications on community compliance during a public health crisis. *Information Systems Research*, 35(2): 505-527.
- Murray, H. A. 1938. *Explorations in personality*. New York, NY: Oxford University Press.
- Niven, K., Hughes, D. J., Tan, J. K., & Wickett, R. 2024. Individual differences in interpersonal emotion regulation: What makes some people more (or less) successful than others?. *Social and Personality Psychology Compass*, 18(4): e12951.
- Niven, K., Totterdell, P., & Holman, D. 2009. A classification of controlled interpersonal affect regulation strategies. *Emotion*, 9(4): 498–509.
- Nozaki, Y., & Gross, J. J. 2025. Bridging supportive communication and interpersonal emotion regulation: An integrative review. *Journal of Social and Personal Relationships*, 42(8): 02654075251335816.
- Nozaki, Y., & Mikolajczak, M. 2023. Effectiveness of extrinsic emotion regulation strategies in text-based online communication. *Emotion*, 23(6): 1714–1725.
- Ogunfowora, B. T., Nguyen, V. Q., Steel, P., & Hwang, C. C. 2022. A meta-analytic investigation of the antecedents, theoretical correlates, and consequences of moral disengagement at work. *Journal of Applied Psychology*, 107(5): 746-755.
- Pauw, L. S., Sauter, D. A., Van Kleef, G. A., Sels, L., & Fischer, A. H. 2025. The dynamics of interpersonal emotion regulation: How sharers elicit desired (but not necessarily helpful) support. *Emotion*, 25(2): 287–299.
- Petriglieri, G., & Maitlis, S. 2019. When a colleague is grieving. *Harvard Business Review*, 97(4): 116-123.
- Petrova, K., & Gross, J. J. 2023. The future of emotion regulation research: Broadening our field of view. *Affective Science*, 4(4): 609-616.
- Pletneva, L. 2024. Turning work into a refuge: Job crafting as coping with personal, grief-inducing events. *Academy of Management Journal*, 67(4): 1055-1083.
- Prause, J., & Dooley, D. 1997. Effect of underemployment on school-leavers' self-esteem. *Journal of Adolescence*, 20(3): 243-260.

- R Core Team 2022. ***R: A language and environment for statistical computing***. Vienna, Austria: R Foundation for Statistical Computing.
- Rack, J. J., Burleson, B. R., Bodie, G. D., Holmstrom, A. J., & Servaty-Seib, H. 2008. Bereaved adults' evaluations of grief management messages: Effects of message person centeredness, recipient individual differences, and contextual factors. ***Death Studies***, 32(5): 399-427.
- Rathje, S., Mirea, D. M., Sucholutsky, I., Marjeh, R., Robertson, C. E., & Van Bavel, J. J. 2024. GPT is an effective tool for multilingual psychological text analysis. ***Proceedings of the National Academy of Sciences***, 121(34): e2308950121.
- Reis, H. T., Clark, M. S., & Holmes, J. G. 2004. Perceived partner responsiveness as an organizing construct in the study of intimacy and closeness. In ***Handbook of closeness and intimacy*** (pp. 211-236). East Sussex, UK: Psychology Press.
- Reis, H. T., Lemay Jr, E. P., & Finkenauer, C. 2017. Toward understanding understanding: The importance of feeling understood in relationships. ***Social and Personality Psychology Compass***, 11(3): e12308.
- Reis, H. T., Sheldon, K. M., Gable, S. L., Roscoe, J., & Ryan, R. M. 2000. Daily well-being: The role of autonomy, competence, and relatedness. ***Personality and Social Psychology Bulletin***, 26(4): 419-435.
- Rogers, C. R. 1957. The necessary and sufficient conditions of therapeutic personality change. ***Journal of Consulting Psychology***, 21(2): 95-103.
- Ryan, R. M., & Deci, E. L. 2008. Self-determination theory and the role of basic psychological needs in personality and the organization of behavior. In O. P. John, R. W. Robins, & L. A. Pervin (Eds.), ***Handbook of personality: Theory and research*** (3rd ed., pp. 654-678). New York, NY: The Guilford Press.
- Ryan, R. M., & Deci, E. L. 2017. ***Self-determination theory: Basic psychological needs in motivation, development, and wellness***. New York, NY: The Guilford Press.
- Sahi, R. S., Gaines, E. M., Nussbaum, S. G., Lee, D., Lieberman, M. D., Eisenberger, N. I., & Silvers, J. A. 2025. You changed my mind: Immediate and enduring impacts of social emotion regulation. ***Emotion***, 25(2): 330-339.
- Sahi, R. S., He, Z., Silvers, J. A., & Eisenberger, N. I. 2023. One size does not fit all: Decomposing the implementation and differential benefits of social emotion regulation strategies. ***Emotion***, 23(6): 1522-1535.
- Settoon, R. P., & Mossholder, K. W. 2002. Relationship quality and relationship context as antecedents of person- and task-focused interpersonal citizenship behavior. ***Journal of Applied Psychology***, 87(2): 255-267.
- Simmons, J. P., Nelson, L. D., & Simonsohn, U. 2011. False-positive psychology: Undisclosed flexibility in data collection and analysis allows presenting anything as significant. ***Psychological Science***, 22(11): 1359-1366.
- Srivastava, S., Tamir, M., McGonigal, K. M., John, O. P., & Gross, J. J. 2009. The social costs of emotional suppression: A prospective study of the transition to college. ***Journal of Personality and Social Psychology***, 96(4): 883-897.
- Stellar, J. E., & Keltner, D. 2014. Compassion. In M. M. Tugade, M. N. Shiota, & L. D. Kirby (Eds.), ***Handbook of positive emotions*** (pp. 329-341). New York, NY: The Guilford Press.
- Stroebe, M. 1993. Coping with bereavement: A review of the grief work hypothesis. ***OMEGA-Journal of Death and Dying***, 26(1): 19-42.

- Stroebe, M., & Schut, H. 2010. The dual process model of coping with bereavement: A decade on. *OMEGA-journal of Death and Dying*, 61(4): 273-289.
- Torre, J. B., & Lieberman, M. D. 2018. Putting feelings into words: Affect labeling as implicit emotion regulation. *Emotion Review*, 10(2): 116-124.
- Tracy, S. J., & Huffman, T. P. 2017. Compassion in the face of terror: A case study of recognizing suffering, co-creating hope, and developing trust in a would-be school shooting. *Communication Monographs*, 84: 30-53.
- Tsai, J. L. 2017. Ideal affect in daily life: Implications for affective experience, health, and social behavior. *Current Opinion in Psychology*, 17: 118-128.
- Yarkoni, T. 2022. The generalizability crisis. *Behavioral and Brain Sciences*, 45: e1.
- Yeomans, M., Boland, F. K., Collins, H. K., Abi-Esber, N., & Brooks, A. W. (2023). A practical guide to conversation research: How to study what people say to each other. *Advances in Methods and Practices in Psychological Science*, 6(4): 25152459231183919.
- Yu, A., Berg, J. M., & Zlatev, J. J. 2021. Emotional acknowledgment: How verbalizing others' emotions fosters interpersonal trust. *Organizational Behavior and Human Decision Processes*, 164: 116-135.
- Zaki, J. 2020. Integrating empathy and interpersonal emotion regulation. *Annual Review of Psychology*, 71: 517-540.
- Zaki, J., & Williams, W. C. 2013. Interpersonal emotion regulation. *Emotion*, 13(5): 803-810.
- Zhang, X., Li, L., Zhang, Q., Le, L. H., & Wu, Y. 2024. Physician empathy in doctor-patient communication: A systematic review. *Health Communication*, 39(5): 1027-1037.